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Global Health and Its Limitations: An Historical Perspective

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ABSTRACT
Humanitarian themes, such as rights and entitlements to universal well-being, feature prominently in narratives of global health, even as many recent authors have pointed to systematic imbalances of power, unfair governance structures, and unwanted influences as evidence of ongoing colonial interference in the health affairs of many low- and middle-income countries. This article employs an historical perspective to analyze major forces that have shaped the development of global health, and which remain as obstacles to its objectives. These include macroeconomics, geopolitics, and the activism and resources of the HIV/AIDS pandemic that led to global health in its current form. Through an examination of this history and its effects, I argue that the humanitarian goals of global health will not be realized without dramatic changes to the field. Particularly in the failure to engage economic relationships and trade policy, global health limits its attention to downstream consequences of resource inequalities, where its goal of a more egalitarian, more healthy world is difficult, if not impossible, to achieve.

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Global Health's Uncertain Boundaries

Global health is a prominent feature of international discourse, representing around $40–50 billion USD annually in official development assistance for health.[1] Yet neither the term nor the field of global health have widely accepted definitions, although they are generally understood to focus on important objectives: health, wellbeing, and humanitarian advancement. Commonly, global health is described with reference to a few issues, including diseases such as HIV/AIDS, malaria, or tuberculosis, or with reference to processes or focus areas, including health financing, human resources for health, or maternal and child health.

However, understanding global health at this level is incomplete because an examination of the constituent pieces does not reveal the forces that have shaped the field nor the larger environment in which it operates. This article adopts an historical perspective emphasizing the external forces that have circumscribed global health and, I will argue, constrained its positive effects. I assess how the origins of the field's primary international agency and academic institutions have influenced its evolution to the present. Similarly, I discuss macroeconomic forces and the HIV/AIDS pandemic as significant influences on the global health of today. This broad, high-level analysis is intended to inform global health academics and professionals about the structures they occupy and illuminate the constraints they face. I argue that engaging and reducing these constraints is necessary to realize the humanitarian potential of global health, meaning to make greater progress toward common goals such as health for all.

A recent systematic analysis by Salm et al.[2] interrogated definitions of global health as captured in their sample of 78 papers.[2] These authors found four thematic areas have been used to define global health: a manifold approach to health improvement taught and advanced at research institutions, an ethically oriented humanitarian pursuit, a form of governance that generates influence by identifying problems and deploying resources across borders, and a "vague yet versatile concept with multiple meanings, historical antecedents, and an emergent future." In this article, I use "global health" in this last sense because it is the most inclusive. As shown in this article, the first of Salm et al.'s themes comes from the legacy of tropical medicine, the second is linked particularly with the HIV/AIDS movement, the third emphasizes political and geopolitical aspects, and the fourth recognizes broad diversity in the past, present, and future, but at the expense of specificity.

This paper traces four major themes that have produced global health as it is today: the collaborative disease control efforts that have been institutionalized

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as the World Health Organization (WHO); the macroeconomic environment; Western colonial and imperial geopolitical interests; and the HIV/AIDS advocacy movements that emphasized rights and entitlements, and brought mainstream engagement and celebrity to bear on human health concerns worldwide. These four themes have shaped global health as it is known and pursued in the West. These themes are generally prominent in the mainstream of global health, although focusing on them excludes perspectives and events from other parts of the world that would be important to a comprehensive account of the field. For example, no country was more important to smallpox eradication than India, and India's success owed much more to domestic actors than those working internationally.[3,4]

Telling this story as the product of larger geopolitical forces, rather than individuals' actions, focuses it on the many political, economic, and ethical forces at play. This permits an emphasis on the conditions and larger interests that have shaped global health (but at the expense of the individuals and personalities that have defined the field from within). A more complete history would include both of these,[5] and many other perspectives, particularly from those communities that have been on the receiving end of global health activities.[6–8] However, the focus of this paper is to analyze how powers greater than the health sector have resulted in a field that is sometimes at odds with its stated humanitarian intentions. Hence, my concern is not a full review of global health's institutions and achievements, but an investigation of some of its limitations.

Many of the limitations discussed in this paper underlie complaints that have become increasingly common in articles discussing the persistence of colonial influences, or coloniality. These include many forms of non-merit supremacy,[9] saviorism,[10] resource inequalities,[11] insufficient fairness,[12] and structural discrimination in research and publishing.[13] These longstanding problems are symptoms and sequelae of the dynamics investigated in this paper.

International Institutions: The Origins of Organized Cooperation in Global Health

This analysis emphasizes how world powers seek to address health issues that occur "between" nations, rather than how they do so within the domestic sphere. Thus, we start with the first efforts to convene Western nations to cooperate on health: the Istanbul Sanitary Court in the 1830s. Delegates gathered from Europe and the Ottoman Empire to discuss cholera, review what was known about the disease, and consider how it might be managed collectively. Those earliest gatherings, convened by the Ottoman Empire, created patterns and challenges that continue to the present day.

First, there was disagreement about the scope of collaboration. The Ottomans envisioned joint action against cholera wherever it was found, whereas the European countries wanted to collaborate only on preventing its international spread. Second, the countries with greater international trade interests opposed any agreement that could be used to disrupt or delay their shipments. France and the UK worried that their ships would be quarantined unnecessarily by Russia, for example. The advantages of collaborating to manage the international transmission of disease and learn collectively were already in tension with the competitive inclinations of most of the powerful nations. This limited the trust needed for collaboration—and sometimes involved health directly as an area of rivalry.[5,14–16] Concerns for trade and economic advantages slowed cooperation on cholera in the nineteenth century, just as they would undermine collective responses to the COVID-19 pandemic in the twenty-first.

Nonetheless, the advantages of collective approaches to international health issues are many, and thus advocates of cooperation persist. The initial Ottoman meetings led to periodic international sanitation conferences held by Europeans, beginning in 1850.[17] Those meetings eventually led to the creation of the health organization of the League of Nations in 1920, which, in turn, became the WHO in 1948.[18]

This legacy of international cooperation among European and Ottoman powers on health, and its eventual institutionalization in the form of WHO, reflects the need for and advantages of joint action to counter cross-border threats, mitigate the spread of infectious diseases, share knowledge, and coordinate policies and actions. These were among the reasons nations began meeting in the nineteenth century. These collaborative motivations, however, co-existed alongside competitive ones. Trade in particular has been enmeshed with sanitary policy for centuries, if not longer—as have allegations of quarantine being used for competitive advantage. Historian Mark Harrison has argued that international health cooperation was driven by its importance to commercial activities, and both were so significant to nations that they were incorporated into statecraft and diplomacy that led to formal sanitary conferences.[19]

Thus, one central aspect of what we now call global health is its roots in the complex mix of national motivations, some of which pull in different directions. Rivalries in trade, geopolitics, diplomacy, science and medicine, and other areas co-exist in some opposition to the benefits of common definitions, shared approaches, and collective management. These global tensions played out negatively during the COVID-19 pandemic, when many nations used economic and scientific advantages to obtain vaccines and declined to share resources until their own needs were fully met.[20–22] Vaccine nationalism proved far stronger than the COVID-19 Vaccines Global Access (COVAX) facility, which was designed to support equitable distribution globally and was immediately undercut when the US and China declined to join. Rich countries negotiated their own purchases with manufacturers.[23,24] Similarly, Oxford University scientists used an estimated 97–99% public funds to identify a vaccine candidate and initially pledged to make it freely available as a result.[25] They later reversed that pledge to partner with a major for-profit pharmaceutical firm instead, after being pressured to do so by the Bill & Melinda Gates Foundation, in part because a commercial partner would be able to manufacture faster.[26,27]

There are positive counterexamples that illustrate the merits of collaborating in and for health.[16] For instance, the successful global campaign to eradicate smallpox represents a triumph of shared interests and coordination,[28] including monumental efforts by India and between opposite sides of the Cold War.[29] The eradication of smallpox in 1979 is perhaps the signature achievement of global health and has led to substantial progress against many other diseases. WHO's first major program was against malaria, a disease it has helped countries battle, with some success, over the past 75 years.[30] WHO has also led programs that have nearly eradicated polio and guinea worm,[31,32] and substantially reduced the global burden of diarrheal diseases,[33] onchocerciasis,[34] tuberculosis,[35] HIV/AIDS,[36] and many others. In recent decades, WHO has also shown great leadership in negotiating the Framework Convention on Tobacco Control,[37] the world's first binding health treaty, and has led initiatives to support Universal Health Coverage.[38] However, the overall pattern reflects the happenstance of collaborative inclinations of powerful countries, rather than an assessment of health or ethical imperatives.

Macroeconomics: Its Influence on Academic Approaches to Global Health

The mixed legacy of achievements and missed opportunities in global health implies ambiguity, or at least variation, in the relationship between health and trade. Although this is true to some extent, health authorities have been unable to fully counter economic influences in most cases. Unfair economic relationships have been recognized as critical obstacles to public health and development for decades. As the United Nations (UN) took shape during and after World War II, Raúl Prebisch[39,40] and Hans Singer[41,42] independently advanced the argument that colonial trade relationships were structurally incapable of producing development and would only increase dependency by colonies on metropoles. Following the 1955 Bandung Conference, the Non-Aligned Movement was founded in 1961 to help developing countries counter colonial and neo-colonial forces and advance the collective interests of members in the United Nations and other international fora.[43] In the 1960s, UNCTAD, the UN trade and development agency, was established to attempt to make globalization more fair, especially for formerly colonized countries.[44] In the 1970s, the World Health Organization articulated the same point in the Alma-Ata Declaration by calling for a "New International Economic Order."[45] In the 1990s, following intense pressure from activists, the Heavily Indebted Poor Countries (HIPC) initiative was launched by the International Monetary Fund and the World Bank to address unsustainable indebtedness as another attempt to manage the sequelae of structural inequalities in the world economy.[45–48] In the 2000s, the Commission on Macroeconomics and Health reached the same conclusion, calling for a new global partnership between the developed and developing countries.[49] But despite a conclusion made repeatedly over the better part of a century, most institutions and many professionals in global health have not even sustained learning and advocacy around the need for trade reform, much less led counter actions. Each generation appears to make the discovery anew, only to be forgotten sufficiently quickly by enough people that the cycle repeats.

Macroeconomic forces dominate the global health agenda, producing conflicts and structures that are rarely resolved or organized in favor of greater health. Political-economic competition among wealthy states via trade negotiations and traditional diplomacy limits cooperation and contributes to problems such as unmanaged refugee crises and climate change, and at the same time, siphons resources away from poor nations and undermines multilateralism.[50,51] An inability to counter the macroeconomic drivers of inequities has had consistent, negative population health impacts.[52]

For global health, the consequence of these macroeconomic forces is a policy space constrained in several ways. First, the secondary status of global health to commercial and national strategic interests discourages engagement with some of the most significant influences on health such as the behavior of powerful nations. Second, with the partial exception of tobacco control,[53] global health advocates have been unable to shape the trade in commercial products. Health advocates seeking either to advance positive possibilities (such as intellectual property rights that go beyond the TRIPS agreement for patents, trademarks, and copyrights)[54] or limit those with harmful potential (such as ultra-processed foods and sugar-sweetened beverages)[55] have often been unsuccessful. Third, the dependence on donor interest means that research and project funding is focused on everchanging priorities of specialists in the donor countries, rather than responding to felt needs in receiving countries. By earmarking contributions, these same donors also undermine WHO's independence.[50] Fourth, the dominance of macroeconomic factors enforces an intellectual ecology in which economic factors have primacy—this is seen in the emphasis on "value for money."[56] Value for money brings a focus that is incompatible with public priorities that do not generate easily measured economic gains, such as pensions and services for the elderly, or improving health systems.

The profound influences of macroeconomic forces and financial power on defining global health priorities are especially evident in academic and private sector engagement. This dynamic grows directly out of colonialism by European nations and similar imperial activities by the US, which created demand for specialized knowledge about "tropical" diseases and other health obstacles to conquest. Initially, this demand came from the desire to protect European military personnel, who were unable to survive long enough in most of Africa to conquer territory, extract resources, or establish inland trading opportunities.[57] Malaria and other fevers were the major obstacles to European and American occupations in Africa and Asia, while yellow fever initially blocked the extension of US power in the Caribbean, and Central and South America.

In the nineteenth century, empirical knowledge to counter these health threats was developed by military physicians serving alongside troops. Alphonse Laveran discovered the malaria plasmodium in 1880 while supporting the French army's slow conquest of Algeria.[58] In the Americas, the demand for new knowledge about tropical diseases was driven by US imperial ambitions. For example, the control of yellow fever was underpinned by research findings of a US Army Commission that proved its mosquito transmission in 1900. Although that etiology had been correctly posited by Cuban physician Carlos Finlay some two decades earlier, the US had not considered yellow fever as a priority disease until the Spanish-American War and the subsequent occupation of Cuba between 1898 and 1902.[59] The US Army Corps of Engineers immediately applied this knowledge of yellow fever transmission and control to another strategic priority: constructing the Panama Canal.[60]

Although the military was the birthplace of the specialty that became known as "tropical medicine," its leading proponents soon organized independently in schools of tropical medicine. These schools emerged around 1900 in the major colonizing nations of Europe, and in the US in connection with imperialism. The first two schools were the Liverpool School of Tropical Medicine and the London School of Tropical Medicine, both founded in 1898 with support from the government and colonial businesses.[61,62] The Liverpool School, for example, was supported by the Elder Dempster shipping company, which transported goods to and from colonized places.[63] The reasons for establishing an academic specialty included perceived advantages in pay and prestige, as well as greater flexibility in serving the private sector.[64]

Tropical medicine as an independent field served both business and political interests by studying health obstacles to military and commercial objectives. Accordingly, tropical medicine was first concerned with health threats to colonial and imperial interests. It later evolved a secondary purpose: engaging Indigenous or native people in the satellite countries in ways that were transactional or extractive. These traditions eventually evolved to become part of international health. The academic legacy that began in Liverpool and London was further exemplified by other European institutions, including the Netherlands' KIT Royal Tropical Institute (founded in 1910 as the Colonial Institute), the Institute of Tropical Medicine in Antwerp (founded in 1906 to address the threat of trypanosomiasis in King Leopold II's Congo Free State), and the School of Tropical Medicine in Lisbon (which was founded along with the Colonial Hospital in 1902 to assist Portugal's colonial aspirations).

In the United States, a parallel process unfolded with emergence of the American Society of Tropical Medicine in 1902. Tulane University's School of Hygiene and Tropical Medicine was founded in 1912 (by a graduate of the London School of Tropical Medicine) with funding from a major American banana company operating in Honduras.[65] Similar specialized departments and schools soon followed, including at Harvard and other US medical schools. Imperial business interests supported these programs—Harvard received early support from the United Fruit Company, international shipping firms, railroad companies connecting plantations and ports in Central America, and the Firestone Rubber Company, which hired Harvard faculty to explore rubber-growing areas of Africa.[66,67] Reflecting on the early years of American tropical medicine, one of its founding figures in 1914 noted the contributions of the new academic specialty: the "development of ... fertile lands rendered possible without the terrible suffering and loss of life which previously resulted from it. Moreover, the hindrance to commerce and to colonization caused by ignorance of the means of combating tropical disease is slowly but surely becoming appreciated by the laymen in general and particularly by business men."[68]

The connections between private industry and tropical medicine also include the involvement of philanthropic institutions established with capital accumulated from these businesses. By far the largest and most important of these was the Rockefeller Foundation (RF), which engaged in public health questions since its founding in 1913. Even before then, the Rockefeller family funded a Rockefeller Sanitary Commission beginning in 1909 to investigate hookworm because of its major economic and societal consequences.[69] When the RF established a special section for international health in 1914, it immediately began working on the leading disease obstacle to American interests: yellow fever.[70] It formed the first RF Yellow Fever Commission in 1916. Several more followed, investing in yellow fever surveys around the world and supporting yellow fever research in many countries.[71]

The Rockefeller Foundation advanced its vision of tropical medicine by investing in what would become leading schools of public health, including Johns Hopkins (1916), Harvard (1922), and the London School of Tropical Medicine (1924). The latter two already existed, but the influence and contributions of the RF changed their missions. Harvard had been operating a school for training health officers since 1913 in a partnership with MIT, which was contributing expertise in water systems and sanitary engineering. As a condition of receiving RF support, Harvard jettisoned its partnership with MIT to focus much more closely on bacteriology, and moved its Department of Tropical Medicine from the medical school into the newly renamed Harvard School of Public Health. Similarly, in accordance with RF's conditions, the London School of Tropical Medicine added "Hygiene" to its name to emphasize environmental sanitation strategies, such as anti-mosquito techniques that had succeeded in the Panama Canal Zone.

Geopolitics: A Driving Force in the Evolution of Global Health

Trade has always been a primary force shaping global health. The institutional roots of contemporary global health in international agencies and Western academia were established in part to support the economic extraction at the core of colonial and imperial activities. But this history is incomplete without consideration of the political aspects. Generally speaking, "geopolitics" refers to the grand strategies of nations that play out through competition for territory, energy, natural resources, and other forms of power. The portion of the twentieth century after World War II was defined by a geopolitical standoff between the US and the Soviet Union called the "Cold War," in which each side waged war by proxy and sought allies and advantages over the other, including by stockpiling atomic weaponry as a mutual deterrent. This contest was highly influential on global health.

During and after World War II, the US and the UK led efforts to create the UN and its specialized institutions as mechanisms to stabilize international trade and diplomacy. The USSR did not initially want to join, fearing that it would be disadvantaged within its governance structures.[72,73] When the USSR did join, it did so reluctantly, first obtaining concessions in the governance structure that allowed it or any other single member of the Security Council to veto any resolution.[74] Colonialism was one of the most contentious issues in the UN's first decade, in part because the US, the UK, and the USSR all disagreed with each other, creating enough uncertainty on the issue among powerful nations that they saw no path to consensus.[75,76] Disagreement on the big questions was matched by disputes over specific policies. The US and the UK generally supported technological and market-oriented positions, while the USSR favored socialist approaches, including community involvement.[77,78]

The ideological gulf between the Cold War superpowers translated into differing programmatic approaches that could be easily identified with one side or the other. In debates at the World Health Assembly, the US prevailed. Accordingly, WHO's first major initiative, the Global Malaria Eradication Program (GMEP), was shaped by American faith in technology, namely the insecticide DDT. The US's preference for market considerations was further reflected in the justification for prioritizing malaria control, a so-called "malaria tax." The malaria tax referred to the extra expense of buying raw materials produced by sick workers in developing countries, and the reduced ability of these workers to purchase finished goods made in the metropoles.[79] In large part, the GMEP implemented by the UN system employed the strategies used by the colonial and imperial powers.[80] Initial malaria control efforts also centered on areas of strategic interest to the US and its allies in the Cold War—so despite having "global" in the name, the GMEP did not include African countries.[5, 81–83] Locations of strategic interest were sometimes selected according to military significance, for example, the areas around air and naval bases in Turkey. The locations targeted also represented part of a "hearts and minds" campaigns in South and South-East Asia.[84–86] (Malaria had already been controlled by the US in many other strategically important areas, such as Cuba, naval bases in the Philippines, and the Panama Canal Zone.[87,88])

The GMEP began to fail in the mid-1960s due to insect resistance to DDT, with several consequences. First, Western donors began shifting their attention to food production and population control programs. The logic of malaria control had been to increase labor productivity. As a fallback position, donors sought to reduce population growth instead. This reveals the prevalent donor perception at the time of economic development as a Malthusian rationing problem. Both tacks —increasing food availability and decreasing population —were intended to improve the resources available per person to forestall unrest in what was then called the Third World.[89–92]

Second, the international community began to focus on "operations research" as a central component of development programming. Weaknesses in monitoring and research support for the GMEP meant that its strategies were not adjusted as resistance to DDT developed, nor were alternatives developed. This interest in expanding research capacity led to the establishment of the Joint UNDP/World Bank/WHO Special Programme for Research and Training in Tropical Diseases, known commonly as TDR.[93,94]

Third, the failure of malaria control efforts corresponded with a weakening of the influence of malariologists and other disease specialists, who had been associated with the US's interests and the Cold War rivalry. Instead, those who favored more holistic interventions and sought more cooperation among countries as a means of advancing health found their influence ascending. By the mid-1970s, this latter group—a loose coalition of the Soviet Union, Christian groups, humanitarian agencies, and others—began to articulate the principles of Primary Health Care (PHC), a movement whose most enduring achievement may be the 1978 Alma-Ata Conference and Declaration.[95,96] The Alma-Ata Declaration focused on the importance of economic and political rights to health. Its supporters attempted to generate enthusiasm for reforming unfair trade relationships, hoping for a pivot from rivalrous colonialism to fairness and equality.

However, WHO's work on a code of marketing for breastmilk substitutes[97] around the same time increased concern among major trade powers that WHO would use its health authority to curtail private industry. Starting in 1981, the US and aligned countries moved from trying to influence WHO's budget to trying to curtail the institution. That year the US delegate to the World Health Assembly announced the Reagan administration's policy of "zero growth," designed to freeze the assessed contributions that underpinned WHO's independence and reorient its activities through earmarking of funds.[98] The result has been a WHO shaped by those nations that can pay to elevate their own priorities and maintain a focus on specific diseases, rather than tackling the social, political, and commercial determinants of health.[99,100] (The success of the WHO Framework Convention on Tobacco Control is a notable exception to this pattern.)

In the Cold War context of a bi-polar world, disease-specific "vertical" programs were generally associated with technical interventions, expert authority, and US perspectives, while PHC was linked to community-centric, non-Western, and sometimes Soviet perspectives. The PHC approach was dominant at Alma-Ata, but the eradication of smallpox in 1979, a vertical program's triumph, helped sway some observers to a middle path called "selective primary health care" (SPHC). This was advanced by the RF, USAID, UNICEF, the World Bank, and others, but was opposed by WHO. After a meeting at the RF's Bellagio facilities, these allies articulated reservations to the WHO-backed PHC stance: "The goal set at Alma-Ata is above reproach, yet its large and laudable scope makes it unattainable in terms of its prohibitive cost and the numbers of trained personnel required."[101]

Their argument for SPHC was that it maximized the effectiveness of the limited resources available for health by focusing on a few important diseases and a few cost-effective interventions. The argument, and the subsequent successes achieved under SPHC against diarrheal diseases and acute respiratory infections, reflected the rising influence of economic theory in the health field (which was later codified in the World Bank's 1993 *World Development Report: Investing in Health).*[102] SPHC proponents argued for making progress against a few defined foes, a path that was feasible to implement and would, they claimed, serve as the core of a full PHC system that could be built over time. This proposition was championed by James Grant, who became the Executive Director of UNICEF in 1980. Although WHO remained in favor of PHC, the SPHC approach gained more support and came to define the bulk of the international health technical and financial assistance provided in the 1980s.[103] Although these activities could have been used to develop health system capacity, no systematic efforts materialized in spite of criticisms of narrow approaches and ongoing advocacy for PHC.[104]

Although mentioned prominently at Alma-Ata, economic and political rights were then largely ignored in development assistance until after the fall of the Berlin Wall in 1989. In part, this neglect reflected a Cold War calculus emphasizing geopolitical alignment, rather than how states treated their own citizens. But after the Cold War, economic and political rights issues were no longer enmeshed in the larger US-Soviet ideological struggle. The disintegration of the Soviet Union left a uni-polar world in which the "alignment" of states was far less important. Democratization and governance projects became prominent in many Western aid portfolios. A growing appreciation for the role of governance in translating development aid into poverty reduction was a common motivation. This was an important factor in the introduction and subsequent popularity of the Sector Wide Approaches (SWAps) and other general budget support mechanisms that blossomed in the 1990s.[105] The mixed results from the market emphasis of the Reagan-Thatcher 1980s, including disastrous structural adjustment interventions and the Washington Consensus approach to aid, also made clear that the market alone would not solve all problems. The focus on governance represented a retreat from pro-market positions,[106] as did the emphasis on public-private partnerships.[107] Weaknesses in governance at various levels, from local to national to global, have drawn attention as constraints to health system functioning.[108] The proliferation of actors focusing on global health also spurred calls for better governance because of the need for coordination and accountability.[109]

Especially after the 9/11 attacks brought donor attention to the problems of failed states and global economic inequity, targeted aid was commonly linked to governance projects. After 9/11, US assistance was increasingly directed to poor people in hopes of preventing scarcity-induced radicalization. The emphasis on safety nets and social protection could be read as a move to enhance democratic participation by the poor,[110] although they were not paired with trade reform or other attempts to rectify underlying economic inequalities. Similarly, there was little discourse about what exactly a failed state was or why it would fail.

HIV/AIDS: Mobilizing People Globally

Around the same time as the fall of the Berlin wall, the HIV/AIDS pandemic began to reshape international activities in health, leading to a change in nomenclature. The term "global health" was not in widespread use before the HIV/AIDS era, but it had largely supplanted "international health" by the early 2000s.[111] The evolution of the field's name reflects its dominant actors. The "tropical medicine" moniker had fallen into disuse in favor of "international health" by the late 1960s and early 1970s. Although the former was faithful to history and had some correspondence to geography, it was a concept from the colonial period. "International health" seemed more appropriate to the new generation of physicians and scientists, among others. It described their activities well. The international health departments in metropolitan schools of public health comprised faculty doing research in poor countries; similarly, NGOs and other organizations, such as the National Council of International Health founded in Vermont in 1972, were focused on work in "third-world" or "developing" countries. International health could also accommodate population control activities, which were a major focus for development agencies in the 1960s but did not have a direct colonial ancestor, as tropical medicine did. The term had a diffusionist implication that matched the assumptions of many: that technology, ideas, or other resources from the former colonial and imperial countries could be used to improve health elsewhere.

These assumptions were questioned by the advocacy groups that succeeded in securing access to treatment for people with HIV, typified first by gay men's groups in New York and San Francisco. Their efforts inspired and influenced the rights-based approach that Jonathan Mann and other actors then began to expand internationally, as the global extent of the HIV/AIDS pandemic became evident. This momentum grew into the Human Rights for Health movement, showing its strength through several international agreements on rights in the early 1990s.[112] Financial resources followed a similar pattern. Private foundations that had been established to operate in the US turned their attention overseas. For example, the Elizabeth Glaser Pediatric AIDS Foundation was established in 1988 to advocate and fund HIV/AIDS research in the US, but by 1999 was leading an international campaign against mother-to-child-transmission with studies in six countries.[113]

The influx of new people and ideas had implications for nearly all aspects of what had been known as "international health." In the post-World War II days of international health, states were the main governance unit, either via bilateral aid agencies or through the multilateral system. However, this orientation was an uncomfortable fit for HIV/AIDS advocates and other new players entering the field, which they preferred to call "global health."[114] In many cases, governments were antagonistic toward groups central to the HIV pandemic—both as patients and advocates—including gay people, sex workers, and injecting-drug users. A history of marginalization of these groups by states and societies elevated rights and legal entitlements as a cornerstone of treatment and advocacy. Whereas public health is an expression of the authority of a state, and tropical medicine was an extension of that power for colonial and imperial purposes, the new global health was born out of civil society activism intent on compelling states to reform attitudes and policies to confront the pandemic. The private sector was an important ally in this struggle, as was private philanthropy. Famous examples included Project (RED), supported by popular singer Bono and others, and donations by Doris Duke and her foundations.

To this new group, "global health" sounded more egalitarian and was strongly preferred over the older terms. In addition to bringing new actors, the movement spawned several new institutions, including Gavi, the Vaccine Alliance (2000), the Bill and Melinda Gates Foundation (2000), and the Global Fund to Fight AIDS, TB, and Malaria (2001). The push for new institutions can be understood partly as the ascendance of redistributive values championed by activists, and also as a reflection of the mix of attitudes they and private philanthropists had toward state authority.[115] In contrast to the UN family institutions governed by states alone, the governing boards of Gavi and the Global Fund included representatives of the groups that powered the HIV movement: patients, non-governmental organizations, the private sector, and private philanthropy. This choice was an articulation of the Denver Principles, established by people living with HIV/AIDS to center affected people and their communities in all aspects of the response.[116] These actors from the HIV/AIDS movement were able to retain some control over the new institutions in this way, but where they wanted governance reform for greater inclusion, some of the richest countries wanted other reforms to concentrate their power more easily than was possible via UN family institutions. At the Global Fund, for example, all member countries are represented on the Board via regional seats, but the five nations that have contributed the most money—France, Germany, Japan, the UK, and the US—are awarded their own seats. These five seats account for a quarter of the 20 voting seats and are difficult to change because they are based on cumulative contributions (meaning the incumbents have a head start dating to 2000).

The success of the HIV movement would be difficult to overstate, even if it remains incomplete and requires qualifications in some areas. Rich country citizens using rich country resources to chase their aspirations in developing countries has been a longstanding pattern.[117] But the old international health was so overshadowed by the HIV movement that specialists in several of its main focus diseases rebranded their preferred maladies as "Neglected Tropical Diseases" to reestablish their influence in the new era.[118,119]

The activists and funders interested in HIV were less connected to colonialism than those in international health, but they were not completely free of the legacy. Their main linkages with that history were through the culture of saviorism it had promoted,[10,120,121] rather than via academic or professional pedigree,[122] and through the massive resource inequalities it had created.[123–127] The new global health still drew on resources controlled from the metropolitan countries, and was spent largely on citizens and products from these places, even if the benefits were ostensibly for native and Indigenous people in developing countries.

The origin in rich countries shaped the HIV movement and the new global health in programmatic ways as well. In rich countries, HIV had been considered a relatively isolated problem—there were functioning health systems and legal systems strong enough to compel actions by unwilling governments, at least in most areas. Activists focused on the societal, legal, financial, and administrative barriers that were standing between patients and the medicines they needed and wanted. The legal and humanitarian rights arguments that came to typify global health had their roots in places where rights could be upheld based on a court order and the availability of affordable drugs was partly a matter of domestic regulatory change. While access and delivery were intuitive concepts, overcoming stigma and discrimination were central concerns. Investing in scientific and medical research was an important accompanying pillar.

As these obstacles were defeated in rich countries—especially in areas with more resources—the global movement maintained the same playbook, choosing to focus almost exclusively on HIV, and the legal, social, and economic barriers that had been important in the US and other wealthy countries. These choices were not wrong, but they were incomplete when applied in places deprived of resources by long-term extraction. The need for better health systems, more patient education, general access to health care overall, meaningful economic opportunities, and trust between citizens and government under a good social contract were addressed minimally. The focus on civil society and rights was less helpful in places where the government would not cede legal ground, would not comply with attempts to enforce legal entitlements, or did not have the resources to afford foreign-made medicines in the quantities needed, even as activists continued to secure lower and lower prices. Pharmaceutical approaches were initially less helpful with populations trained by a history of betrayal not to trust Western products, and countries whose boundaries, governments, and economies had all been established for extractive purposes were a context so different for the new global health advocates that their significance was not always appreciated.

However, the prices of drug therapies were among the most significant obstacles in rich countries and remained so in the global movement. Perhaps the legacy most influential on the rest of global health have been inclusive collaborations to raise resources, lower prices, boost innovation, and ensure access to medicines, diagnostics and other technologies. When the International AIDS Conference was held in South Africa in 2000, antiretroviral therapies cost $10,000–15,000 USD per patient per year, but one year later a combination of advocacy, collaboration, public pressure, and generic competition had brought the same figure to just $350 USD. This success was one motivation for founding Unitaid in 2006, which is supported mainly by a tax on airline tickets in sponsoring countries. Unitaid then launched the Medicines Patent Pool in 2010 to further streamline generics manufacturing.[128]

Product development partnerships more generally were inspired by the success of HIV activists, including the Foundation for Innovative New Diagnostics in 2003, which was concerned initially with developing low-cost, reliable tests to detect tuberculosis.[129] The Drugs for Neglected Diseases initiative was launched the same year to support the development of therapies for NTDs.[130] More generally, the influence of HIV activism can be seen in the organization of partnerships to solve market failures that affected vaccine and medicine availability, such as Gavi, the Vaccine Alliance.[131]

The resources and the independence of the HIV movement masked some of the limitations in the older international institutions, but large-scale disease outbreaks, such as Severe Acute Respiratory Syndrome (SARS) in 2003 and more recently Covid-19, have made visible the importance of functioning governance mechanisms in health.[132] The coronavirus pandemic dramatically highlighted many shortcomings in existing arrangements.[20] Although the HIV/AIDS era brought a significant humanitarian emphasis to global health, via the inclusion of civil society and patient groups, there are still grounds for debate about the ability of these "newer" participants to contest more entrenched forms of power.

For the most part, therefore, health considerations remain secondary to the prevailing economic conditions that explain historic wealth distributions and dictate ongoing global resource flows, such as the trade interests of powerful nations.[133] This bias was recently revealed by the difficulties faced in regulating commercial products with significance for health,[134] superpower countries' unwillingness to establish supranational authority for the WHO,[135] the failure of the COVAX mechanism to provide fair access to COVID-19 vaccines, and the selfish approaches taken by rich countries in the COVID-19 pandemic.[21,22]

Conclusion

This discussion of nearly 200 years of global health history casts the field as constrained by multiple strands: colonial priorities, academic ambitions, business opportunities, and geopolitics. There is no meaningful way to establish the relative importance of these factors at any given time, but all of them are consequential. And all of them share a common denominator. Each area is characterized by a resource inequality, whether real or perceived, that leads to power inequalities—and these have yet to be confronted by global health practitioners and the nations that support them. No matter how well-meaning individuals may be, when they work within the mainstream of global health they operate within these constraints.

Discussion has begun on some of the sequelae, such as inequalities in authorship and publishing opportunities, inequalities in access to conferences and travel opportunities, and inequalities in the relationships between Northern and Southern academic and non-governmental institutions engaged in global health. These inequalities run counter to the humanitarian and health motivations cited by many people in the profession, but there are much more serious consequences beyond these.

Some seven decades after colonized countries gained political independence, inequalities persist even in areas such as capacity building, infrastructure, and medical research, which should be straightforward to rectify with several generations of effort. The failure to do so demonstrates a need for global health to engage more explicitly with sources of historic inequality. Why the global health research enterprise remains concentrated in elite northern institutions is difficult to explain without comparison to the extractive industries that remain headquartered in the same places. This reality points to the urgent need to democratize knowledge generation both geographically and by Indigenous ways of knowing. Similarly, unfair trade relationships that disadvantage formerly colonized places and commercial products with health consequences should draw the attention of global health professionals as critical obstacles to equity and well-being.

Many of the persistent inequalities in global health relate to more general resource inequalities, as I have argued with reference to macroeconomic forces. Similarly, the distribution of power at WHO remains centered largely around the rich countries of Western Europe and North America. The colonial heritage of tropical medicine finds common expression in global health research and the interventions it produces. Although HIV/AIDS activists were able to make some progress in reshaping the governance of global health, their collaborative visions have yet to transform most areas of the field or counterbalance the vast inequalities in health and other resources that divide the former colonizing and imperial nations from the places and people they once claimed as their own. The refusal or inability of global health to consider the wider picture of international economic and political inequalities suggests that most of its practitioners are content to remain as saviors, even if the cost includes a world organized to produce the victims they purportedly rescue.

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